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Larinova

Investor memo
The OPD assistant for Indian doctors.

FOR PRIVATE CIRCULATION · DILIGENCE-GRADE

PREPARED FOR	Mr. Sundar Venkatraman, CEO · APAR Innosys LLP, Chennai
BY	Gabriel Antony Xaviour, Founder & CEO · Larinova Private Limited
SUBJECT	Lead-angel cheque proposal — ₹75 Lakh for 10% equity at ₹7.5 Cr post-money
ENTITY	Larinova Private Limited · CIN U62012TN2026PTC192444 · Tamil Nadu
DATE	29 April 2026

SECTION 1

Executive summary

India has 1.1 million doctors and one AI clinical scribe — Eka.Care, which serves only English and Hindi. Larinova is the alternative: the only end-to-end OPD platform for Indian doctors that speaks all 22 official Indian languages from day one, at roughly 10% under Eka.Care's price for a fuller product (booking, AI-guided patient intake, a 60-second pre-consult Prep Brief, a real-time clinical scribe in every Indian language, and a wellness follow-up agent). The product is live in production, used by 5 alpha doctors as of today, and priced at ₹999 per doctor per month — roughly 10% under Eka.Care, the only adjacent competitor in India, which itself supports only English and Hindi and ships only the scribe layer.

Larinova Private Limited was incorporated on 22 April 2026 (CIN U62012TN2026PTC192444). The founder, Gabriel Antony Xaviour, is a 22-year-old Loyola-ICAM engineering graduate, holds 90% of the company, and is full-time on Larinova. His father Antony Xaviour holds 10% as a co-founding governance shareholder — he runs Rax Tech, the family's ₹20–40 Cr/year IoT and e-surveillance business, and APAR Innosys is a longstanding Rax Tech client. Sundar Venkatraman therefore knows the Xaviour family's product-and-execution standard from existing commercial dealings.

Gabriel is asking for **₹75 Lakh in exchange for 10% equity at a ₹7.5 Cr post-money valuation**. The cheque funds 22 months of runway, the first strategic hire by month 9, a hospital business-development motion across Chennai and a controlled second-engine launch in Indonesia. The committed milestone at month 12 is 500 paying doctors at ~₹50 Lakh ARR with line of sight to break-even — the proof needed to raise a Series Seed at ₹25–50 Cr post-money, a 5–10× markup on Sundar's entry valuation.

Why this opportunity is unusual

- **The only Indic-native clinical AI in production — all 22 Indian languages, day one.** ~420,000 Indic-language doctors (Tamil, Telugu, Bengali, Marathi, Kannada, Malayalam, and beyond) have no AI EMR alternative today. Larinova is built on Sarvam AI's Indic models, designed for clinical context across the full Indian language family — not phonetic translation.
- **The only full-OPD-stack platform in the Indian market.** Eka.Care does scribe; Practo does booking; Healthplix does clinic management. Nobody else combines them. Larinova replaces 5 disconnected tools with one.
- **Capital-efficient unit economics.** 85% gross margin, sub-1-month CAC payback, ₹999/month per seat. Profitability per doctor begins on day 30.
- **Founder track record:** 70+ hackathon wins in four years, prior production work at RPS AI (San Francisco; Orange DAO + Delphi Ventures + TRIVE Singapore + UF Venture backed) and Marlin Protocol (Dubai).
- **Pre-existing trust.** Sundar already transacts commercially with the Xaviour family. The risk on character and execution is reduced before the cheque is written.

SECTION 2

Problem & market

The clinical workflow problem

An Indian outpatient doctor sees 30+ patients per day. After every consult, they must produce a SOAP note, an ICD-10 code, and a prescription — typed manually into an EMR or written by hand. Most EMRs are designed in English. When patients describe symptoms in Tamil, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati or any of India's other official languages, the doctor performs mental translation while typing, and clinical nuance is lost in transit. The cumulative documentation overhead averages 90 minutes per doctor per day — either at the cost of patient time, or of going home on time. Existing AI scribes partially solve documentation in English and Hindi, but completely fail to serve the Tamil-, Telugu-, Bengali-, Marathi-speaking — and 18 other Indic-language — practices that make up the majority of clinical India.

The market

India has approximately 1.1 million doctors. Larinova serves all of them from day one — the platform is native to all 22 Indian languages via Sarvam AI. Eka.Care, the only adjacent AI scribe in India, has reached approximately 65,000 doctors as of 2025 — almost entirely English- and Hindi-speaking. That leaves roughly a million doctors unserved by AI scribing, including the entire Indic-language population across Tamil, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati, Odia, Assamese, and beyond. Larinova's Day-1 launch cohort is Chennai-based Tamil doctors (a tactical choice driven by founder geography), but the platform addresses every Indian doctor from the first paying customer.

SEGMENT	DOCTORS	CURRENTLY SERVED BY	LARINOVA ROLE
Tamil (TN, Puducherry, KA fringe) — Chennai launch cohort	**~140,000**	Nobody — English EMRs only	**Live day 1. Founder home advantage.**
Telugu / Bengali / Marathi (AP, TS, WB, MH)	**~280,000**	Nobody	**Live day 1 — same product, native via Sarvam**
Hindi-speaking (UP, MP, BR, RJ, DL)	**~390,000**	Eka.Care (scribe only)	**Live day 1 — competing on full OPD + price**
English-primary (urban metros)	**~300,000**	Eka.Care, Practo, Healthplix	**Live day 1 — competing on full stack + multi-language flexibility**
Total India addressable	**~1.1M**	**Larinova serves the entire market from day one**	

Revenue ceiling math: at ₹999/month per doctor, India's full 1.1M-doctor market is ~₹13,000 Cr/year in revenue ceiling. The 420K underserved Indic-language doctors alone — those with no AI EMR alternative today — represent ~₹500 Cr/year. Capturing just 5% of that underserved segment is ~₹25 Cr ARR, without competing for a single doctor that has another option. This is the kind of market where modest execution still produces a meaningful business.

The Indonesia opportunity (the second engine)

Indonesia has approximately 180,000 doctors and zero direct AI scribe competitor. Local clinic management software (Klinik Pintar, Assist.id, Trustmedis) costs Rp 250K–3M/month for scheduling and records, but none of them auto-generate clinical notes. Global premium scribes (Nuance, Suki, Abridge) cost $200–$830 per doctor per month and have no Bahasa Indonesia support. Larinova at IDR 299K/month (~$18) is structurally uncatchable on price and language simultaneously. Larinova's landing page is already live in Bahasa at larinova.com/id; ₹8 Lakh of this round is allocated to Indonesia entry as proof-of-cross-border, not as a full second launch.

SECTION 3

Solution & product

Larinova is one platform that handles the entire outpatient journey — from the patient booking the slot to the wellness follow-up seven days after the consult — built natively in every Indian language from day one.

The five layers

1. **Booking.** Patients book a consultation slot from a public page on the doctor's domain. They self-confirm by email; the doctor's calendar updates instantly.
2. **AI-guided intake.** Before the appointment, the patient fills a structured form. An AI agent follows up for missing details and accepts lab uploads, so the doctor walks in with complete information.
3. **60-second Prep Brief.** The moment the patient arrives, the doctor sees a one-screen brief: presenting symptoms, prior medications, flagged red flags, and a structured chronology drawn from prior visits and intake responses.
4. **Indic-native consultation scribe — all 22 Indian languages.** The doctor speaks naturally in any Indian language — Tamil, Hindi, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati, Odia, Assamese — English, or any mix. Powered by Sarvam AI's full Indic clinical model family, Larinova generates a SOAP note, an ICD-10 code, and a draft prescription in real time. The doctor reviews, edits, and signs in under two minutes.
5. **Wellness follow-up agent.** A conversational agent checks in with the patient on day 1, day 3, and day 7. Replies are classified — only flagged ones reach the doctor. Routine recovery never bothers the doctor.

Technology infrastructure

Larinova is built on Next.js with Supabase as the data layer, deployed on Vercel. Real-time multi-Indic transcription (all 22 official Indian languages) runs on Sarvam AI's WebSocket streaming API. Clinical reasoning and SOAP-note generation route through a dedicated Claude Service infrastructure (claude.fierypools.fun) which centralises and caps unit cost across our AI inference. Asynchronous workflows (intake, dispatch, follow-up) are orchestrated through Inngest, a durable event-driven queue. The product is a Progressive Web App — works on every doctor's phone or tablet without an install — with offline fallback wired via Serwist. ABDM and ABHA integration are native; the company is DPDP Act 2023 compliant by design.

> **Why this stack matters to an investor:** capital-efficient, modern, vendor-neutral. We do not pay platform rent to a competitor (no Microsoft Health Bot, no Google Cloud Healthcare API). Our AI inference is layered behind our own service, so we can swap models without product changes. This means our gross margin compounds as model costs fall.

SECTION 4

Where we stand today & 12-month roadmap

Today (Day 7 since incorporation)

Larinova was incorporated as a Private Limited company on 22 April 2026. We deliberately present small, real numbers rather than inflated metrics — Sundar's eventual fact-check is the only audit that matters, and the honest version is strong enough.

- **5 doctors** are actively onboarded as alpha pro users in Chennai (Tamil-speaking — first cohort by geography, not platform limitation). Co-building with the founder weekly.
- **2–3 hospitals** are in active onboarding conversations, expected to deploy within 30 days. These are multi-doctor pilots — the enterprise wedge.
- **Full product is live in production** at app.larinova.com — booking, AI intake, Prep Brief, Indic-native scribe (22 languages), follow-up agent.
- **Pricing is locked** at ₹999/month or ₹9,990/year on the Pro tier. Free tier capped at 20 consultations/month for top-of-funnel.
- **Multi-Indic clinical AI is operational** across all 22 official Indian languages via Sarvam AI's Indic models — the only such deployment in production in India today.
- **Cap table is clean:** Gabriel 90% / Father 10%. Zero outside money. Zero advisor equity. Zero SAFE notes outstanding. Bala (Balachandar Seeman) is on a commission-only ambassador structure with no equity.
- **Current account & GST registration are in flight** — expected to complete before the funded share allotment.

The 12-month roadmap

HORIZON	COMMITMENTS
Month 1–3	50 paying Chennai doctors · 3 Chennai clinic deals · first hospital pilot signed · ambassador program live with Bala + 2 founding ambassadors · Indonesia landing testing on first 5 leads
Month 4–6	120 paying doctors · 2 hospital deployments · ~₹1.4 Lakh MRR · Indonesia first 5–10 paying doctors · referral loop active from Month-1 cohort
Month 7–9	250 paying doctors · 5 hospital deals signed · expansion beyond Chennai (Coimbatore, Madurai) · first strategic hire (BD or engineering)
Month 10–12	500 paying doctors · ~₹50 Lakh ARR · break-even line of sight · Series Seed conversation initiated at ₹25–50 Cr post-money — 5–10× markup for Sundar

> **Why these numbers are conservative.** 500 paying doctors out of India's 1.1 million is less than 0.05% market share — well within reach for a focused Chennai-first motion with one operator and a working ambassador program. The plan does not require unusual luck; it requires executing on a live product in a market with no full-Indic competitor.

SECTION 5

Business model & unit economics

Larinova is a per-doctor monthly SaaS subscription. The model is simple, the gross margin is high, and the CAC payback is under one month — the unit economics of a category-leading software business.

Pricing

- **Free tier:** 20 consultations per month. Designed to let new doctors experience the full product without cost friction.
- **Pro tier:** ₹999 per doctor per month, or ₹9,990 per year (a ₹1,998 saving). Unlimited consultations.
- **Hospital tier (custom):** ₹3 Lakh – ₹12 Lakh annual contract value depending on doctor count (15–60 doctors), billed quarterly.

Unit economics

METRIC	VALUE	NOTE
ARPU per doctor (Pro tier)	**₹11,988 / year**	Or ₹9,990 on annual prepay (~₹833/month effective)
Cost to serve per doctor	**~₹150 / month**	Sarvam STT + Anthropic LLM + Supabase + Vercel; all variable
Gross margin	**~85%**	Inference cost falls as models commoditise — margin compounds
CAC (ambassador channel)	**~₹4,000**	₹1,500 activation + ₹2,500 paid conversion to Bala / ambassadors
CAC payback	**< 1 month**	Recovered within first paid month at ₹999 monthly billing
Path to ₹1 Cr ARR	**~840 doctors**	Achievable in 14–18 months with the funded GTM motion
Path to ₹10 Cr ARR	**~8,400 doctors**	~0.8% of India's 1.1M doctors. Year 3 with hospital-tier mix.

> **Industrial-CEO test:** at 85% gross margin, ₹999 monthly per seat, sub-1-month CAC payback, and a hospital tier that compounds revenue per relationship, Larinova has the unit economics of a category-leading SaaS business — even before we layer on enterprise hospital deals. Profitability per doctor begins on day 30 of paid use.

SECTION 6

Competition & moat

Larinova has one indirect competitor in India (Eka.Care), a fragmented adjacent market in Indonesia (clinic-management tools without AI scribe), and a set of premium global scribes (Nuance, Suki, Abridge) who do not operate in Indian or Indonesian markets and are priced for US healthcare systems. None of them deliver what Larinova delivers, in any market, at any price.

DIMENSION	LARINOVA	EKA.CARE (INDIA)	NUANCE / SUKI / ABRIDGE (US)
Languages	**All 22 Indian languages live day-one (Tamil, Hindi, Telugu, Bengali, Marathi, Kannada, Malayalam, Punjabi, Gujarati, Odia, Assamese, +) via Sarvam**	English + Hindi only	English only
Product scope	**Full OPD: booking, intake, prep, scribe, follow-up**	Scribe only	Scribe only
Patient booking	**Yes — public booking page**	No	No
Pre-consult AI	**60-sec Prep Brief on every patient**	No	No
Wellness follow-up agent	**Day 1, 3, 7 conversational**	No	No
ABDM / ABHA integration	**Full native**	Partial	Not applicable
Price per doctor / month	**₹999**	~₹1,100	₹17,000 – ₹70,000

Why the moat is structural

Eka.Care has chosen to ship English and Hindi for three years. Adding even one additional Indic language is not a feature flip — it requires integration with Sarvam's full Indic model family, retraining on clinical corpora across each language, and clinical-domain validation per language. Larinova ships all 22 Indian languages from day one. We expect at least 12–18 months of head-start on every non-Hindi Indic market. By the time Eka.Care could match our language coverage, we will have the Indic-language doctor relationships, the ambassador network, the hospital pilots, and the regulatory integrations locked in.

Global scribes face a different barrier: they price for US healthcare economics and do not have the local presence, regulatory integration (ABDM/ABHA), or Indic language coverage to compete in India. Their

entry would require a complete repricing and re-architecture — not a rational priority for them given their US growth runway.

The most credible competitive risk is not from a competitor but from **doctor adoption speed**. Indian doctors are habit-bound; even the right tool takes time to displace manual notes. Our mitigations are co-building with our 5 alpha doctors weekly to drive the product to obvious-better, the free trial to remove cost friction, and the ambassador program to create peer pull.

SECTION 7

Team, cap table & family context

Founder

Gabriel Antony Xaviour, 22, is the founder and CEO of Larinova. A graduate of Loyola-ICAM (LICET, Chennai) with a Bachelor of Engineering in Electronics and Communication, he has spent the last four to five years as one of India's most prolific competitive product builders — winning 70+ hackathons across AI, Web3, and consumer software while shipping production code for three companies of note. As Founding Engineer at **RPS AI** (San Francisco), he helped build a company backed by Orange DAO, Delphi Ventures, TRIVE Singapore, and UF Venture, which has raised millions across early rounds. As Solutions Engineer at **Marlin Protocol** (Dubai), he worked on trusted execution environments and privacy compute infrastructure. He has also worked inside **Rax Tech** (Chennai) — the IoT, software, and e-surveillance company his parents founded and run today, currently doing ₹20–40 Cr/year — the same Rax Tech that supplies APAR Innosys.

He is solo on Larinova, full-time, and AI-augmented. He builds both the technical product and the commercial motion himself, with Claude as a force multiplier across engineering, sales, and operations. He intends Larinova to be the operating system of every Indian hospital and, eventually, every hospital globally — the most valuable healthcare company India has produced. He has chosen to dedicate the next twenty years to this goal.

> "The biggest battle in technology right now is healthcare AI. I want to win it from India."

Family context

Gabriel's father, **Antony Xaviour** (DIN 01646014), is the second subscriber on the Memorandum of Association and holds 10% as a co-founding governance shareholder. He is the CEO of Rax Tech in his own right and has no operational or advisory role in Larinova — his shareholding is structural, not active. Gabriel's mother is the CTO of Rax Tech. Larinova draws on the family's entrepreneurial standard but operates entirely independently. APAR Innosys's existing commercial relationship with Rax Tech is the relationship through which this conversation began — Sundar already knows the family delivers; he is now evaluating whether the next-generation product warrants his backing on its own merits.

Cap table — pre and post round

HOLDER	ROLE	PRE-ROUND	POST-ROUND
Gabriel Antony Xaviour	Founder & CEO	**90.0%**	**81.0%**
Antony Xaviour	Co-founding shareholder (governance only)	**10.0%**	**9.0%**
Sundar Venkatraman	Lead angel — new this round	**—**	**10.0%**
Total		**100.0%**	**100.0%**

Sales channel: Balachandar Seeman is Larinova's independent BD partner on a structured ambassador commission of ₹1,500 per activation, ₹2,500 per paid conversion, and a ₹25,000 bonus per 5+ doctor clinic deal. He holds no equity. There are no advisor grants, SAFE notes, convertible notes, or other equity-linked instruments outstanding.

SECTION 8

The ask, use of funds, risks & closing

The ask

₹75 Lakh in exchange for 10% equity in Larinova Private Limited, at a **₹7.5 Cr post-money**
valuation. Clean equity — no SAFE, no convertible note, no liquidation preference. Funds land into the Larinova current account once the company's authorised capital is increased from the current ₹1 Lakh to ₹10 Lakh via a routine SH-7 filing with the MCA (~7–10 working days). We are ready to execute the share allotment within seven days of agreed terms.

Use of funds

BUCKET	AMOUNT	WHAT IT PRODUCES
Founder runway (₹1L/mo × 22 months)	**₹22 L**	Founder stays full-time, no income distraction
Hospital BD travel + Bala commissions	**₹10 L**	10–15 hospital contracts signed across Chennai cluster
First strategic hire (month 9–12)	**₹12 L**	BD or engineering lead — eliminates founder bus-factor
Indonesia market entry	**₹8 L**	2 trips to Jakarta, 5–10 paying ID doctors as cross-border proof
Marketing + content + brand video	**₹6 L**	50+ inbound doctor leads/month by month 12
AI inference + infra	**₹6 L**	Capacity for 200+ active doctors at full Pro usage
Compliance + legal	**₹4 L**	GST, DPDP, HPR, MSME, ToS, SH-7 — investable corporate state
Tools + subscriptions	**₹3 L**	Claude, Cursor, Linear, Notion, Vercel Pro
Buffer for unknowns	**₹4 L**	DLT template fees, Meta WhatsApp BSP, surprises
Total	**₹75 L**	**22 months runway · path to ₹50 L ARR**

Risks & mitigations

RISK	MITIGATION
Eka.Care expands beyond English/ Hindi	We are 22 Indic languages live day one vs their 2; per-language clinical training is non-trivial; deeper ABDM moat + full OPD stack they don't have; Indic-language clinic relationships locked in by then

RISK	MITIGATION
Solo founder bus-factor	This raise funds the first strategic hire by month 9; AI augmentation already de-risks single-person execution; cap table is clean if a successor is needed
Doctor adoption slow / habit-resistant	Co-build with 5 alpha doctors weekly; free trial removes cost friction; ambassador program creates peer pull from trusted Chennai doctor network
Indonesia regulatory delays	India is primary market; Indonesia is upside not load-bearing; ₹8 L Indonesia allocation is bounded
AI infra costs scale faster than revenue	~85% gross margin even today; Claude Service caps unit cost; free vs Pro tier controls runaway usage; model costs are falling industry-wide

Why Sundar, why now

This cheque comes from a relationship that already exists. APAR Innosys has trusted Rax Tech for years — Sundar already knows the Xaviour family delivers. Larinova is the next chapter from the same family, with measurably better unit economics (85% gross margin SaaS vs. hardware + services), a 50× larger addressable market (1.1M Indian doctors vs. India's e-surveillance buyer base), and no incumbent to displace in the full-Indic segment. Sundar gets first-mover position in the Larinova cap table before the Series Seed conversation begins.

We are ready to send a term sheet for review within 48 hours of Sundar's expressed interest, execute the SH-7 filing in parallel, and close the share allotment within seven days of agreed terms. The full company documentation set — Certificate of Incorporation, MoA, AoA, DIN approvals, PAN, TAN, and recent MCA filings — is available on request.

> **Direct line to founder:** Gabriel Antony Xaviour · gabriel@larinova.com · WhatsApp +91 84386 11643 · larinova.com · Larinova Private Limited, CIN U62012TN2026PTC192444, 4/2377 Sithalapakkam, Tambaram, Kanchipuram, Tamil Nadu 600126.